

DENTO-ALVEOLAR TRAUMA ASSESSMENT PRO FORMA

For Dental Core Trainees | Based on IADT Guidelines 2020 | DCT / SHO / On-call Use

Affix Patient Label

Patient Name:                      DOB / Age:                      NHS No:

Date/Time of Injury:                      Date/Time Seen:                      Clinician:

1. HISTORY OF TRAUMA

Mechanism:   ☐ Fall                      ☐ RTA                      ☐ Assault                      ☐ Sport                      ☐ Collision                      ☐ Other:

Place:   ☐ Home                      ☐ School                      ☐ Work                      ☐ Road                      ☐ Sports facility                      ☐ Other

Triage:   ☐ LOC? Y / N   ☐ Duration:                      ☐ Vomiting Y/N   ☐ Amnesia Y/N   ☐ Headache Y/N   ☐ GCS:

Tetanus status:   ☐ Up to date                      ☐ Not up to date                      ☐ Unknown                      Referred by: ED / GDC / Self / Other

2. MEDICAL HISTORY - AMPLE

Allergies:                      Meds:                      PMH:

Bleeding tendency / Anticoagulants Y/N   ☐   Smoker Y/N   Alcohol Y/N   Pregnancy Y/N   ☐   Social: Smoking/alcohol

3. DENTAL HISTORY

Prev trauma Y/N   Ortho Tx Y/N   GDP regular Y/N   Immunocompromised Y/N

Tooth fragment accounted for? Y/N   If avulsion: storage medium & extra-oral time:

4. EXTRA-ORAL EXAMINATION

Facial symmetry: Normal / Asymmetry   ☐ Lacerations                      ☐ Abrasions                      ☐ Haematoma                      ☐ Swelling

Site:                      TMJ: Opening / Deviation / Click / Pain                      ☐ Lymph nodes: Palpable / Tender

Cranial nerves: I-XII intact? Y/N   Details:

Rule out # - tenderness: Zygoma / Frontal / Mandible / Maxilla - Y/N   ☐   Sensory deficit Y/N

5. INTRA-ORAL EXAMINATION - SOFT TISSUES

☐ Lips                      ☐ Labial mucosa                      ☐ Buccal mucosa                      ☐ Gingiva                      ☐ Tongue                      ☐ Floor of mouth                      ☐ Palate

Details (laceration size, depth, foreign body, active bleeding):

Occlusion: Normal / Deranged / Step / Open bite / Premature contact - Details:

6. HARD TISSUE / PERIODONTAL ASSESSMENT - CHART: 17-27, 47-37

|    |    |    |    |    |    |    |    |    |    |    |    |    |    |    |    |
|----|----|----|----|----|----|----|----|----|----|----|----|----|----|----|----|
| 18 | 17 | 16 | 15 | 14 | 13 | 12 | 11 | 21 | 22 | 23 | 24 | 25 | 26 | 27 | 28 |
|    |    |    |    |    |    |    |    |    |    |    |    |    |    |    |    |
| 48 | 47 | 46 | 45 | 44 | 43 | 42 | 41 | 31 | 32 | 33 | 34 | 35 | 36 | 37 | 38 |
|    |    |    |    |    |    |    |    |    |    |    |    |    |    |    |    |

For each injured tooth complete:

| Tooth | Fracture type | Mobility 0-3 | Displacement | Tenderness to percussion | Vitality test | Colour |
|-------|---------------|--------------|--------------|--------------------------|---------------|--------|
|       |               |              |              |                          |               |        |
|       |               |              |              |                          |               |        |
|       |               |              |              |                          |               |        |
|       |               |              |              |                          |               |        |

Fracture key:   ☐ None   ☐ Enamel infraction   ☐ Enamel   ☐ Enamel-dentine   ☐ Complicated crown   ☐ Crown-root   ☐ Root   ☐ Alveolar

Displacement key:   ☐ None   ☐ Concussion   ☐ Subluxation   ☐ Lateral   ☐ Extrusive   ☐ Intrusive   ☐ Avulsion   Mobility: 0=none 1=<1mm horiz 2=>1mm horiz 3=vertical

7. INVESTIGATIONS

☐ Periapical(s) of:                      ☐ Upper standard occlusal                      ☐ Lower occlusal                      ☐ OPG                      ☐ CBCT indicated? Y/N

☐ Clinical photo Y/N   ☐ Sensibility test: Cold / EPT                      ☐ Fragment accounted for Y/N   Other:

## 8. DIAGNOSIS - IADT CLASSIFICATION (TICK ALL THAT APPLY PER TOOTH)

### Hard tissue & pulp:

- ☐ Infraction ☐ Enamel # ☐ Enamel-dentine # ☐ Complicated crown # (pulp exp)  
☐ Crown-root # ☐ Root # ☐ Alveolar #

### Periodontal:

- ☐ Subluxation ☐ Extrusive luxation ☐ Lateral luxation ☐ Concussion  
☐ Intrusive luxation ☐ Avulsion

### Soft tissue:

- ☐ Laceration ☐ Contusion ☐ Abrasion

## 9. MANAGEMENT - ACUTE

Analgesia given: \_\_\_\_\_ LA: Type / amount: \_\_\_\_\_ Abx required? Y/N Type: \_\_\_\_\_

### Fracture management:

- ☐ Smoothing / Comp restoration / Fragment rebond ☐ Pulp cap (CaOH / Biodentine) ☐ Pulpotomy  
☐ RCT extirpation & dressing ☐ Repositioned & splinted ☐ Avulsed tooth replanted Extra-oral time: \_\_\_\_\_

**Splint:** ☐ None ☐ Flexible (orthodontic wire + composite) ☐ Rigid Teeth splinted: \_\_\_\_\_

Duration planned: 2wk / 4wk / other \_\_\_\_\_ ☐ Soft tissue closure: sutures Y/N type/no: \_\_\_\_\_

## 10. ADVICE GIVEN & DISCHARGE

- ☐ Soft diet ☐ Avoid biting with injured teeth ☐ CHX 0.12% mouthwash BD 1wk ☐ Oral hygiene instruction  
☐ Analgesia advice ☐ Abx advice ☐ Tetanus advice ☐ Head injury advice given + leaflet Y/N ☐ Avoid contact sports  
☐ Dental Trauma leaflet given Y/N ☐ GDP informed Y/N Patient info: <https://www.dentaltraumaguide.org>

## 11. FOLLOW-UP PLAN & REFERRALS

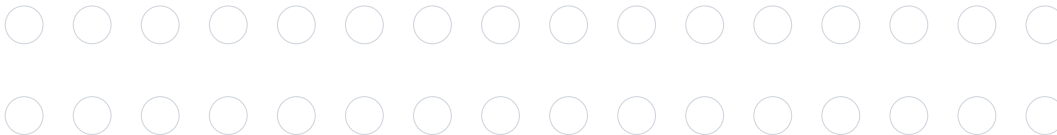
Review: ED / OMFS / Paeds / Ortho / Restorative / GDP

Splint removal date: \_\_\_\_\_ Vitality re-test: \_\_\_\_\_ Radiograph follow-up: \_\_\_\_\_

Follow-up timeline per IADT: ☐ 2wk ☐ 4wk ☐ 6-8wk ☐ 3mo ☐ 6mo ☐ 1yr (note: root # and luxation need extended)

## 12. CLINICAL NOTES / DIAGRAM

Draw injuries, fracture lines, displacement direction, lacerations...



Clinician Name / Grade / Signature: \_\_\_\_\_ Supervisor / Consultant: \_\_\_\_\_

Date: \_\_\_\_\_ Time: \_\_\_\_\_ GMC/GDC No: \_\_\_\_\_

This pro forma does not replace clinical judgement. Always refer to IADT 2020 guidelines and local SOPs. Consider safeguarding if NAI suspected.